

THE PICTURE — what the report will be, what it answers, where it goes

WC/2024/227 · 15 August 2026 · the closing position after the day's work

PART 1 — WHAT IT WILL LOOK LIKE

One report. One attendance. Roughly 8 to 15 pages, in the register of his report of 13 February 2025. Fourteen questions across three matters, built on four attachments — three of which the other side wrote.

Section	Content
Opening	First seen 24 October 2024 , continuous care since; what he reviewed; what he was instructed to assume
Matter 1 1.1–1.5	Diagnosis, framework, criteria met and when · onset · differentials excluded · background: the 16 November 2023 entry <i>and the temazepam prescribed the same day</i> , and the respondent's asserted 2022 anxiety and ADHD history, which is outside the span of the records he holds · premorbid features from his own assessment · the complete source list
Matter 2 2.1–2.3	The conditions of the work, quoted from the role description and the Chief Executive's letter — whether and how such conditions can contribute · fatigue and the rostering of 17–18 March 2024, with the MET calls and the leave of 19 March · the administrative sequence : the August 2023 rostering error, the February 2024 leave decline where the attachments were in fact present, and the May 2024 pay correction
Matter 3 3.1–3.3	Reasoning before conclusion. Other factors placed in time from his own file — the partner present and protective on 24 October, the breakdown running December to February, and bereavement dated at last · then the interval, answered as a continuing exposure · then causation, expressly built on both: a significant contributing factor
Matter 3 3.4–3.7	Prognosis and current capacity · the adjustments in functional terms, with exacerbating conditions, complaint handling and working memory · the requirements of the position in four parts · foreseeable risk
Closing	■ Whether the opinion depends on his account · ■ whether anything in the records qualifies it

PART 2 — WHAT IT ANSWERS: BOTH INSTRUMENTS, ONE DOCUMENT

The Notice of Non-Party Disclosure states the matters in issue in three numbered lines. Those are the report's spine — so it answers the Regulator's own framing, in the notice she served on his practice.

The Request for Medical Information asks nine questions. **Seven are answered outright, one on his terms, and two are declined by design.**

Their question	Answered at	
1(a) when was MDD first diagnosed	1.1	ok
1(b) clinical basis for the three named stressors	2.1–2.4 + 3.3	new
1(c) self-report, own assessment, or other?	Part B + Part D(c)	key
1(d) foreseeable risk and controls	3.8	ok
2 lawful direction · conduct discussions	3.7	new
3 fit under existing reporting arrangements	—	declined
4 restrictions, basis, duration, review	3.5	ok
5 what complaint handling encompasses	3.5(b)	ok
6 inherent requirements <i>without restrictions</i>	3.6(a)–(d)	on his terms
7 working memory, in functional terms	3.5(c)	ok
8 exacerbating tasks and environments	3.5(a)	ok
9 if we cannot accommodate	—	declined

■■■ **1(c) is the one that pays.** They offered three options — self-report, his own assessment, or other medical information. **The answer is a fourth: documents authored by the employer and facts admitted by the Regulator.** They wrote the question that produces it.

■ **The three declined are the three that could hurt** — all workplace questions dressed as medical ones. Part D declines them expressly and offers to say so if asked.

PART 3 — HOW IT IS POSITIONED

What it is: a treating psychiatrist's report, obtained by the patient, answering the matters in issue in his proceeding, and provided to his employer in answer to its request for medical information. **One report, two recipients, two purposes, both declared.**

Funded by Metro South Health, which required the information. ■ That makes the causation opinion harder to attack, not easier — it cannot be said to have been bought by him. ■ It is **instructed by him and addressed to him**; the covering email states both on its face.

To	The covering note does
The Regulator	■ One line. Continuing disclosure, and that the same report has gone to the employer. ■ No argument, no commentary, no request — a report argues for itself, and anything added subtracts from it
The employer	It answers the request of 31 July · the scope line , identifying which questions are workplace rather than medical · their own policy — unjustifiable hardship tested against the whole organisation , onus on Queensland Health · the ask: implement the adjustments, restore pay · the 3 August questions, folded in · and that the Regulator holds the same report

Sent simultaneously. No sequencing inference available to anyone. ■■ And each is told the other holds the identical document — one sentence that forecloses any suggestion of two versions or selective provision.

PART 4 — WHAT IT CHANGES

	No report	■ With it
Appeal resolves favourably	30%	62%
Pay restored / return ~3 months	30%	51%
Neither outcome	49%	20%

■ **Structured judgement, not measurement.** Read the movement, not the decimals.

On the appeal: it moves from *unproved on causation* to *proved on the medical, contested on the characterisation* — the only contest the Regulator has left, and the one where the Chief Executive's letter hurts her. **On the employment track:** it removes “*we cannot accommodate*” and puts the adjustments into a framework whose onus is theirs.

PART 5 — ■ WHAT IT STILL DOES NOT DO

1. **It does not decide the appeal.** s 32(5)(a) remains argument on an agreed factual base, and Taylor and Reese have not given oral evidence.
2. **It does not reach the 2026 wages.** Only the employment track does — and the deemed refusal of **24 August** is the live route there.
3. **It may not say what is hoped.** He declined once already. The instruction asks him to say where the material is insufficient rather than qualify — so a “*cannot say*” returns as a clean answer, not a damaged one.
4. ■ **Question 3.6 is still short of its best evidence** — the three movement forms and the April 2025 call statistics are not in the pack.